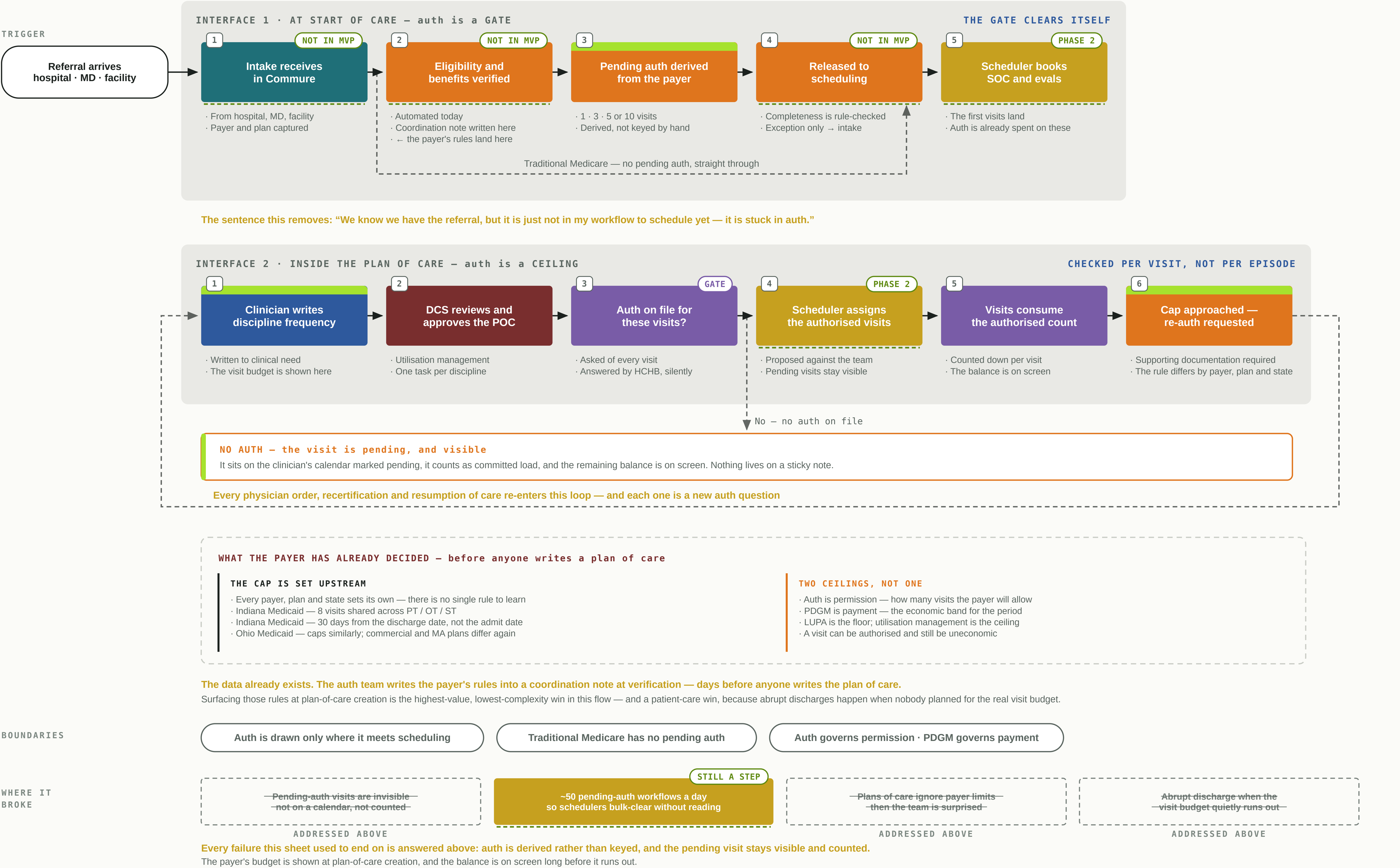


Authorization

Read beside Flow-Authorization — same blocks, same positions. The tool shows; it does not act. A dashed green edge is a later phase



1

Clinician writes discipline frequency

· Written to clinical need
· The visit budget is shown here

2

DCS reviews and approves the POC

· Utilisation management
· One task per discipline

3

Auth on file for these visits?

· Asked of every visit
· Answered by HCHB, silently

4

Scheduler assigns the authorised visits

· Proposed against the team
· Pending visits stay visible

5

Visits consume the authorised count

· Counted down per visit
· The balance is on screen

6

Cap approached — re-auth requested

· Supporting documentation required
· The rule differs by payer, plan and state

CHECKED PER VISIT, NOT PER EPISODE

GATE

PHASE 2

No — no auth on file

NO AUTH — the visit is pending, and visible

It sits on the clinician's calendar marked pending, it counts as committed load, and the remaining balance is on screen. Nothing lives on a sticky note.

Every physician order, recertification and resumption of care re-enters this loop — and each one is a new auth question

WHAT THE PAYER HAS ALREADY DECIDED — before anyone writes a plan of care

THE CAP IS SET UPSTREAM

· Every payer, plan and state sets its own — there is no single rule to learn
· Indiana Medicaid — 8 visits shared across PT / OT / ST
· Indiana Medicaid — 30 days from the discharge date, not the admit date
· Ohio Medicaid — caps similarly; commercial and MA plans differ again

TWO CEILINGS, NOT ONE

· Auth is permission — how many visits the payer will allow
· PDGM is payment — the economic band for the period
· LUPA is the floor; utilisation management is the ceiling
· A visit can be authorised and still be uneconomic

The data already exists. The auth team writes the payer's rules into a coordination note at verification — days before anyone writes the plan of care.

Surfacing those rules at plan-of-care creation is the highest-value, lowest-complexity win in this flow — and a patient-care win, because abrupt discharges happen when nobody planned for the real visit budget.

BOUNDARIES

Auth is drawn only where it meets scheduling

Traditional Medicare has no pending auth

Auth governs permission · PDGM governs payment

WHERE IT BROKE

—Pending-auth visits are invisible—
—not on a calendar, not counted—

ADDRESSED ABOVE

~50 pending-auth workflows a day
so schedulers bulk-clear without reading

STILL A STEP

—Plans of care ignore payer limits—
—then the team is surprised—

ADDRESSED ABOVE

Abrupt discharge when the
—visit budget quietly runs out—

ADDRESSED ABOVE

Every failure this sheet used to end on is answered above: auth is derived rather than keyed, and the pending visit stays visible and counted.

The payer's budget is shown at plan-of-care creation, and the balance is on screen long before it runs out.

VISUALISE ONLY · MVP, PROPOSED · a green stripe = the tool shows it; a dashed green edge = a later phase; purple = HCHB · phase 1 is visualisation only (DE-03)

Flow 3T · authorization — visualise only — mvp